03 — Screening Protocol (Standard Operating Procedure)

Purpose: Every eligible patient is screened the same way, and every positive screen gets the same response.

1. Eligibility

Rule	Definition
Logged visit	Every visit by a patient ≥ 18 for any clinical encounter. Log **all** adult visits, not only screened ones, because they form the denominator for P1.
Screen **due**	No complete screen in the prior **90 days**
Screen not due	Record `screen_status = not_due`. The visit still counts toward P1 (up to date).
Excluded from denominator	Patient medically unstable / transferred to ED; patient unable to participate (e.g., acute intoxication, severe distress). *Document the reason and retry at the next visit.*
Patient declines	**Stays in the denominator** (counts as not screened) but is recorded as `declined`. Decline rate is a balancing measure.

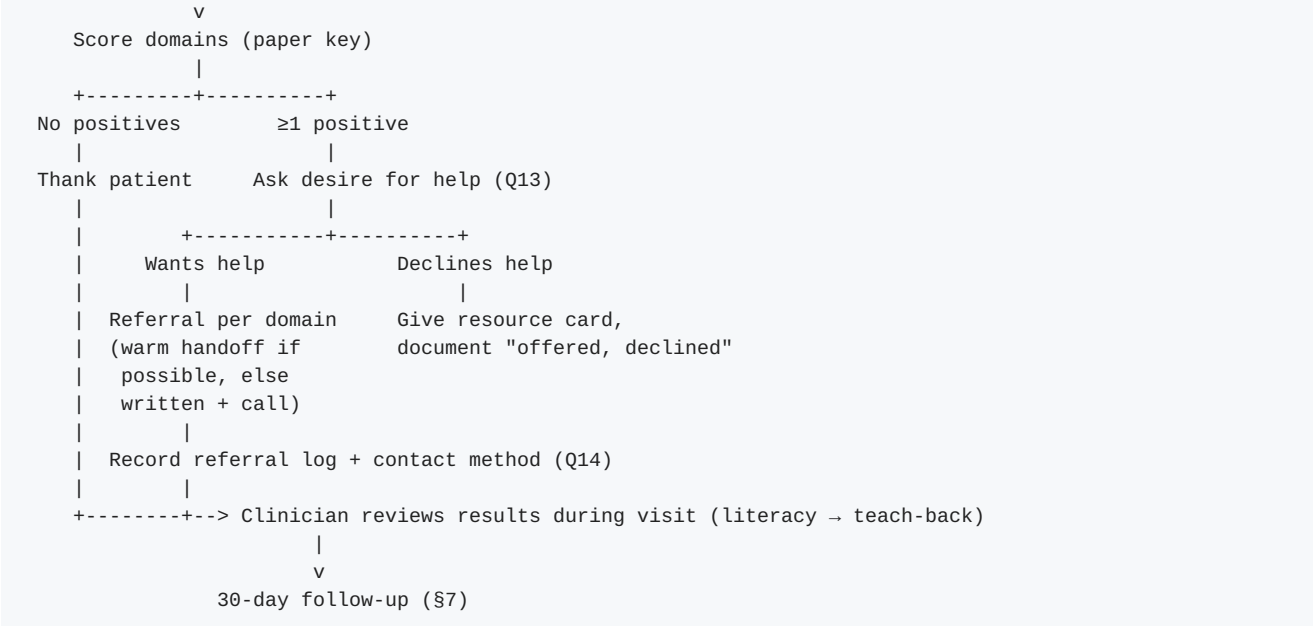
Why 90 days: living situation and needs change quickly in this population. Annual rescreening (common in primary care) would miss changes. Adjust the interval if the clinic's visit frequency differs.

2. Roles

Step	Primary	Backup
Check whether screen is due	Intake/registration staff	Screening champion
Administer screen	MA / RN / CHW (trained)	Clinician
Review positives + ask desire for help	Person who screened	Clinician
Warm handoff / referral	Referral owner (CHW / case manager)	Screener
Safety-positive response	Clinician (licensed)	Clinic lead
30-day follow-up	Referral owner	Screening champion
Data entry (logs)	Screener (same day)	Project lead (weekly audit)

3. Workflow

```
Arrival --> Intake: screen due? (log / flag)
               | no ------------------------------> usual visit
               | yes
               v
         Private space available? (needed for safety items Q7-Q10; if not, mark deferred)
               |
               v
     Administer Q1–Q14 (read aloud)  -- patient declines -> record "declined", offer resource card
               |
```



4. Administration standards

- Read questions **exactly as written**, without paraphrasing the scored items. A brief clarification is OK after the scripted question.
- Use a professional interpreter (phone/video) for non-English speakers. Do not use family or friends as interpreters for safety items.
- Use the intro script from 02.
- Record answers on the paper form, then transcribe to the screening log the same day.

5. Responding to positive screens (non-safety)

Domain	First-line response	Referral target (see 04)
Housing: unsheltered / shelter / vehicle	Warm call to shelter intake / Coordinated Entry; hypothermia program Dec–Mar	Fairfax OPEH / shelters / Hypothermia Prevention Program
Housing: at risk of losing housing	Eviction-prevention referral	Coordinated Services Planning (CSP)
Housing quality	Referral to CSP	CSP
Food	Same-day food if available; pantry referral; SNAP application help	WFCM; DFS (SNAP)
Transportation	NEMT info if Medicaid; reduced-fare programs	Cardinal Care NEMT; TOPS / TRIP
Utilities	Energy assistance referral	DFS Energy Assistance; CSP
Health literacy	**Clinical response:** teach-back, verbal instructions, pictograms, plain-language handouts	(no external referral)
ID/documents	Help with application; fee-waiver eligibility	VA DMV / Vital Records (free if < 21 and homeless); CSP

Referral types (record one): `warm_handoff` (staff calls or introduces the patient during the visit), `direct_referral` (staff sends the referral), `information_only` (written resource given).

6. Safety pathway (HITS ≥ 11, or any disclosure of current danger)

1. Thank the patient for sharing. Make sure the patient is in a private space.
2. Clinician assesses **immediate danger**. If the patient is in imminent danger, call **911**.
3. Offer the **Fairfax County Domestic and Sexual Violence 24-Hour Hotline: 703-360-7273 (TTY 711)**. Offer to call with the patient.
4. Do **not** give written materials that could put the patient at risk if found. Ask first.
5. **Mandatory reporting:** Virginia requires certain professionals to report suspected abuse, neglect, or exploitation of adults aged 60+ or incapacitated adults to Adult Protective Services (Va. Code § 63.2-1606). **Confirm reporting obligations with the clinic's compliance lead before go-live** (see 08).
6. Document per clinic policy in the clinical record. Record only `safety_positive = Y` and the referral in the QI log, with no narrative detail.

7. Follow-up (closed loop)

- At **14 ± 7 days**, and again by **30 days** if needed, the referral owner contacts the patient using the Q14 method (phone, shelter or case manager, or the next clinic stop).
- Ask: "Were you able to connect with ___? Did you get what you needed?"
- Record the `followup_status`: `connected`, `not_connected`, `unable_to_reach`, `declined_followup`, or `pending`.
- If the patient was not connected, re-refer or remove the barrier, for example with a bus fare or a call made together.

8. Rescreening

- Rescreen every 90 days, or sooner if the patient reports a major change (eviction, new shelter, safety event).

9. Documentation

What	Where (paper-first)	Phase 2
Completed form	Locked binder / scanned to chart	EHR flowsheet
Screening log	`screening_log.csv` (de-identified study ID)	Export
Referral log	`referral_log.csv`	Closed-loop referral platform
Study ID ↔ patient key	**Separate, locked**, held by clinic only	—